

Northwell Preliminary Modified Treatment and Dressing Advisor: v1.0-2026

V1.0-2026 (testing, implementation)

Disclaimer: Intended for simple guidance and reference per WOCN/system/site policy and guidelines. Not intend as replacement for a provider. Northwell does not endorse any one product or service. Each individual patient see individualized approach. Please refer to your WOCN (wound educator, teams)



* Required

1. Wound Etiology

Determining type of wound

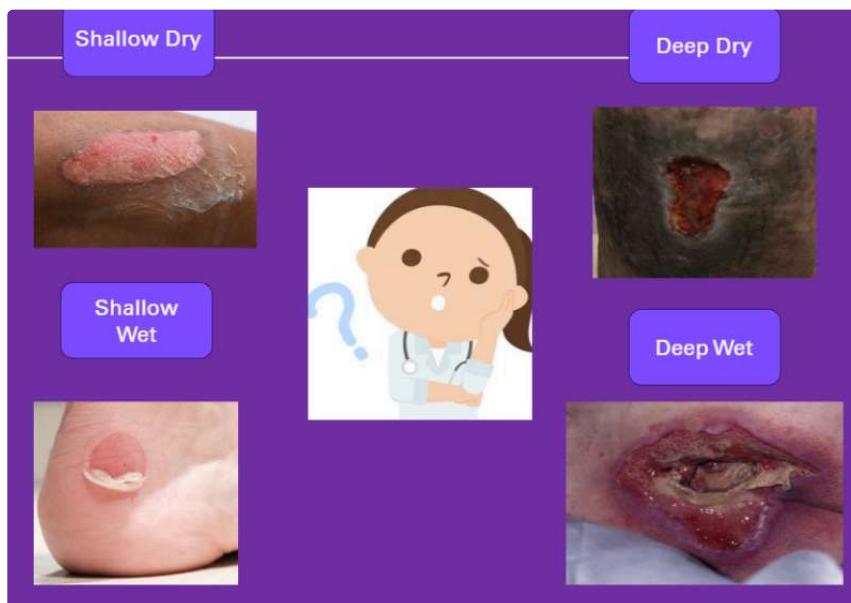
1

What type of wound do you suspect?

- ☐ A. Pressure injury
- ☐ B. Skin tear
- ☐ C. Ostomy related
- ☐ D. Moisture Dermatitis

2. Pressure Injury Assessment

Determine location of your wound (Pressure Injury), and think opposite dressing for what the wound shows (wet/dry, dry/wet). 24 hours AFTER admission, including day of, stage 1-2-3-4-unstageable are considered community acquired (CAPI), 72 hours for DTPI. Everything else is a HAPI, and requires WOCN consult. All guides are reference material only-every individual patient situation may be different and require different dressings.

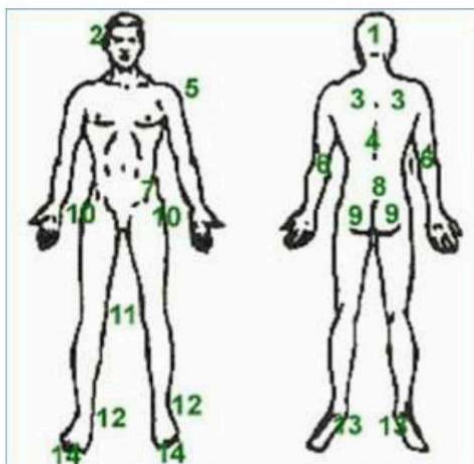


2

Is the area in question over a bony prominence? (typical bony prominence= elbows, scapula, spinous process, sacrum, coccyx, ischium, trochanter, knees, heel, malleolus, tips of toes are ATYPICAL, unless developed within the admission may not require a WOCN consult).

Bony Prominences

- ▶ Occiput
- ▶ Ear
- ▶ Scapula
- ▶ Spinous Process
- ▶ Shoulder
- ▶ Elbow
- ▶ Iliac Crest
- ▶ Sacrum/Coccyx
- ▶ Ischial Tuberosity
- ▶ Trochanter
- ▶ Knee
- ▶ Malleolus
- ▶ Heel
- ▶ Toe



☐ Yes

☐ No

3

You selected that the wound is not over a bony prominence. Please consider WOCN consult, reaching out by teams, or "wound educator" on vocera for clarification.

☐ return to the beginning of the form

☐ end the form

4

You selected bony prominence. What stage do you believe the wound is?



☐ stage 1



☐ stage 2



☐ Stage 3



☐ stage 4



☐ Unstageable



☐ sDTPI

5

Selected stage 1 PI. Is the area of redness blanching?

☐ Yes

☐ No

6

Stage 1 PI. Selected "yes" to blanching. This may not be a pressure injury! But, this may present an area at risk for PI development. consider reaching out to WOCN if a possilbe HAPI, otherwise apply cation, contact layer of adaptic/mepitel one if needed for fragile skin, and then cover with allevyn foam dressing IF pt is not at risk for incontinence in this region of the body. If AT risk, consider barrier ointment (critic aid). Consider combining these solutions with turning and offloading.

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☐ go to end of form.

7

Stage 1 PI. Non blanching was selected. Is the wound over an area at risk for incontinence like sacrum/coccyx, or over extremities/hips
?

☐ area at risk for incontinence

☐ area NOT at risk for incontinence

8

Stage 1 PI, wound is at risk for deteriorating due to incontinence. Please consider: barrier ointment (critic aid), turning and offloading with isolation of incontinence.

☐ return to beginning of form

☐ end form

9

Stage 1 PI, wound is not at risk for deteriorating due to incontinence (extremity, limb, away from buttocks). Consider a hydrocolloid (comfeel dressing) changed twice weekly, or cation and foam dressing (if skin is fragile to break), or cation and adaptic or mepitel one covered by foam dressing (if skin is not fragile to break)

☐ beginning of form

☐ end of form

10

Selected stage 2 PI. Is the wound an intact blister, ruptured blister, or open shallow lesion?



☐ intact blister



☐ ruptured blister



☐ open shallow lesion

11

Selected Stage 2 intact blister. Is the area over the buttocks, or a limb/lateral hip

☐ buttocks

☐ limb/lateral hip

12

Selected intact blister stage 2 over buttocks. Consider for no further risk of incontinence (saline cleanse, xeroform, foam dressing), or risk for incontinence (triad and foam dressing if able to obtain good seal, otherwise OTA)

☐ return to beginning of form

☐ end of form

13

Selected intact stage 2 blister over limb/lateral hip. is it a large blister that may likely rupture, or a small blister likely to be maintained?



☐ large blister



☐ small blister

14

Stage 2 large intact blister. If over a joint region and or likely to burst, consider surgical consult for lancing and draining. To leave blister intact, use caviol, non adherent layer such as adaptic or mepitel one silicone, secured by 4x4 non woven gauze, then secured with light wrap of kerlix.

☐ return to beginning of form☐ end of form

15

Stage 2 small intact blister over limb. Consider hydrocolloid dressing (comfeel) changed twice weekly, or caviol with adaptic or mepitel one covered by foam dressing changed every other day.

☐ return to beginning of form☐ end of form

16

stage 2 ruptured blister selected. Is the blister over the buttocks or limb/lateral hip

☐ buttock area☐ limb/lateral hip

17

Stage 2 ruptured blister over buttocks region is selected. triad and foam dressing if able to obtain good seal, otherwise OTA. triad is a "touch up" job, do not scrub off dried triad.

☐ return to beginning of form☐ end of form

18

Stage 2 ruptured blister over limbs/lateral waist selected. consider hydrocolloid for SMALL blister, or saline cleanse then caviol then xeroform cut to size then covered by non woven gauze/abd pad and kerlix change once daily

☐ return to beginning of form☐ end of form

19

Stage 2 PI open lesion is selected. is it over the buttocks or limbs/lateral hips?

- ☐ buttocks region
- ☐ limbs/lateral hips

20

Stage 2 PI open lesion over the buttocks. consider triad covered by foam dressing IF able to obtain a secure seal. otherwise, leave OTA.

- ☐ return to beginning of form
- ☐ end of form

21

Stage 2 PI open lesion over the limbs/lateral hips consider hydrocolloid for SMALL blister changed twice weekly, or xeroform and foam dressing changed once daily.

- ☐ return to beginning of form
- ☐ end of form

Stage 3-4-Unstageable PI selected, for shallow dry wound. Use the below infographic as a basic guide. Shallow dry wounds need hydration to promote autolytic debridement, and an insulating cover to retain that moisture and bodily temperature. In body areas concerned for incontinence and contamination, consider a dressing cover that will obtain a good SEAL and lock out urine/stool, sweat, etc. for extremities and limbs, consider fragility of patient skin for using adhesive dressings. For general debridement, santyl is used when the wound is not infected, and medihoney is used when infection may be present. Alternative to medihoney is santyl WITH dakins solution 1/4 strength. Santyl may be impeded when used with metal ions, avoid using santyl with ag+ unless specified. Honey may be used with silver.

SHALLOW DRY WOUNDS

Hydration:	Explanation:	Cover With:	Explanation:
	Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal. foam dressing for gentle adhesive agitated wound bed, use contact layer
	Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above
	Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above.
	Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI	N/A	N/A

- ☐ return to beginning of form
- ☐ end of form

Stage 3-4-unstageable PI. Shallow wet wounds require some moisture to continue autolytic debridement, but a way to wick excess fluid from the wound bed.

Hydrofibers (aquacel extra for non infected wounds or ag+ for infected wounds) act as that "candle wick", once wet this dry dressing turns into a "gel" to minimize pain with removal* while it is not ideal to cover a wet wound with dry gauze thereby forming a "wet to dry", non woven gauze provides less trauma and shedding of components into the wound base. Using as a contact layer over intact skin may be acceptable to reduce adhesive risk.

hydrocolloid dressings help to wick away excess moisture, while maintaining an optimal environment for healing. May cause excess maceration if the wound bed is highly exudative. Moreso for skin tears.

For general debridement, santyl is used when the wound is not infected, and medihoney is used when infection may be present. Alternative to medihoney is santyl WITH dakins solution 1/4 strength. Santyl may be impeded when used with metal ions, avoid using santyl with ag+ unless specified. Honey may be used with silver.

SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for atraumatic changes. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal. foam dressing for gentle adhesion
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for atraumatic changes. Consider moderate drainage wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal. foam dressing for gentle adhesion
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal. foam dressing for gentle adhesion
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI	N/A	Serves as contact layer and covers wound

- ☐ return to beginning of form
- ☐ end of form

Stage 3-4-Unstageable PI. Deep dry wounds require donated moisture to effectively begin autolytic debridement, and wound cleaning. We also need to pack (lightly) to wick away developing drainage, and promote granulation "taking away the negative space". For general debridement, santyl is used when the wound is not infected, and medihoney is used when infection may be present. Alternative to medihoney is santyl WITH dakins solution 1/4 strength. Santyl may be impeded when used with metal ions, avoid using santyl with ag+ unless specified. Honey may be used with silver.

	Hydrate:	Why:	Pack With:	Why	Cover With:	Why
DEEP DRY WOUNDS		Donates moisture to wound base, promotes autolytic debridement		Use Non-woven for clean wet wounds, woven for when we need to debride.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Donates moisture to wound base, promotes autolytic debridement. Pulls moisture from deep in wound.		Same as above		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Enzymatic debridement, loosens necrosis in conjunction with packing.		Same as above		Same as above
		General isotonic cleanser. Donates moisture		Same as above		Same as above

- ☐ return to beginning of form
- ☐ end of form

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Stage 3-4-Unstageable PI. Deep wet wounds require packing of the negative space, but also a way to wick away excess fluid from the wound base. A drowning wound is not a happy wound. For general debridement, santyl is used when the wound is not infected, and medihoney is used when infection may be present. Alternative to medihoney is santyl WITH dakins solution 1/4 strength. Santyl may be impeded when used with metal ions, avoid using santyl with ag+ unless specified. Honey may be used with silver.

	Wick / Dry:	Why:	Pack With:	Why	Cover With:	Why
DEEP WET WOUNDS		Draws moisture off the wound bed. Gels on contact with moisture for atraumatic changes. Consider for infected wounds	Aquacell, and gauze PRN 	Wicking action, good for tunneling		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal foam dressing for gentle adhesive
		Draws moisture off the wound bed. Gels on contact with moisture for atraumatic changes. Consider moderate drainage wounds	Aquacell, and gauze PRN 	Wicking action, good for tunneling		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal foam dressing for gentle adhesive
		Use Non-woven for clean wet wounds, woven for when we need to debride.	N/A	N/A		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal foam dressing for gentle adhesive
		Suction will pull away moisture, promote granulation, and contain wound environment	Foam already present in dressing	Contained system. Can use additional supplies as directed		Self contained environment. Can use barrier rings and stoma paste to achieve seal on difficult wet wound edges. Use Cavilon!

- ☐ return to beginning of form
- ☐ end of form

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Stage 3-4 PI. What classification fits your wound:

- ☐ deep dry
- ☐ deep wet
- ☐ shallow wet
- ☐ shallow dry

27

unstageable PI. What classification fits your wound:

- ☐ deep dry
- ☐ deep wet
- ☐ shallow wet
- ☐ shallow dry

28

sDTPI. What classification fits your wound:

- ☐ shallow dry (or flat bruise appearance, non blistered, skin intact)
- ☐ shallow wet (evolving DTI, skin break, or bloody blister)

29

sDTPI shallow dry wound selected. May be present as flat discolored lesion, skin is intact.

- ☐ is the area over buttocks/perineal?
- ☐ is the area over limbs, torso, back?

30

sDTPI shallow dry over the buttocks selected, that may be present as intact skin and discolored lesion.
Please select from the following options

- ☐ I can obtain a good seal with a allevyn foam dressing, pt is not at risk for incontinence.
- ☐ I can obtain a good seal with a allevyn foam dressing, pt is at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is at not at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is at risk for incontinence

31

sDTPI shallow dry over the limbs/torso/back selected, that may be present as intact skin and discolored lesion. Consider the following options: paint with cavilon, cover with contact layer (adaptic, mepitel one silicone), then secure with foam dressing allevyn OR abd pad and kerlix , OR island telfa dressing and change QOD. peel back daily for skin inspection. Alternatively, if not at risk for maceration, consider hydrocolloid dressing changed once weekly.

☐ return to beginning of form

☐ end of form

32

sDTPI shallow dry wound over the buttocks/perineal area at risk for incontinence, you selected "can obtain a good seal with allevyn foam dressing, pt is not at risk for incontinence". Consider: cavilon, contact layer of adaptic/mepitel one silicone, and cover with allevyn foam dressing, changed once every other day, and peeled back daily for inspection.

☐ return to beginning of form

☐ end of form

33

sDTPI shallow dry wound over the buttocks/perineal area at risk for incontinence, you selected "can obtain a good seal with allevyn foam dressing, pt is at risk for incontinence". Consider: cavilon, contact layer such as mepitel one/silicone, covered by allevyn foam dressing, changed once daily and as needed. We need to be careful with foam dressing placement particularly here, as incontinence and moisture can get trapped behind the foam dressing and worsen the wound bed.

☐ return to beginning of form

☐ end of form

34

shallow dry wound over the buttocks/perineal area at risk for incontinence, you selected "cannot obtain a good seal with allevyn foam dressing, pt is at not risk for incontinence". Consider: critic aid clear ointment BID and with each episode of incontinence. Be sure to offload effectively and consider z flo/fluidized blue pillow and or waffle cushion to remove pressure from affected area. We would avoid a foam dressing in this scenario because of the risk for further maceration, that can be caused by an improper fitted foam dressing.

☐ return to beginning of form

☐ end of form

35

shallow dry wound over the buttocks/perineal area at risk for incontinence, you selected "cannot obtain a good seal with allevyn foam dressing, pt is at risk for incontinence". Consider: baza ointment BID and with each episode of incontinence, offloading and turning per protocol, and if wound degrades consider triad.

- ☐ return to beginning of form
- ☐ end of form

36

sDTPI wet (evolving with skin break, or bloody blister) selected

- ☐ is the area over buttocks/perineal?
- ☐ is the area over limbs, torso, back?
- ☐ is the area over the heels or foot specifically?

37

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER THE BUTTOCKS selected. Which more resembles the current stage of your wound?



- ☐ now becoming an unstageable



- ☐ remains flat and otherwise dry



- ☐ partial opening, but cannot definitively re-stage



- ☐ skin loss is present, but cannot effectively re-stage, and/OR tissue remains deep purple

38

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an unstageable. please select from the following options

- ☐ I can obtain a good seal with a allevyn foam dressing, pt is not at risk for incontinence.
- ☐ I can obtain a good seal with a allevyn foam dressing, pt is at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is not at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is at risk for incontinence

39

sDTPI wet (evolving) over the buttocks, now appearing as unstageable PI. Selected can obtain a good seal with a foam dressing, pt is not at risk for incontinence. Consider: saline cleanse, adding wound gel (hydrogel or honey) to wound base, cavilon to periwound, secured by allevyn foam dressing. The MD will need to order a santyl once daily wet to moist dressing change. Please message/call WOCN for updated note. Surgical debridement may be necessary.

- ☐ return to beginning of form
- ☐ end of form

40

sDTPI wet (evolving) over the buttocks, now appearing as unstageable PI. Selected can obtain a good seal with a foam dressing, pt is at risk for incontinence. Consider: saline cleanse, cavilon to periwound, apply wound gel to wound base (hydrogel or honey), secured by foam dressing. The MD will need to order santyl once daily dressing changes. Please message/call WOCN for updated recs and note. Surgical debridement may be necessary.

- ☐ return to beginning of form
- ☐ end of form

41

sDTPI wet (evolving) over the buttocks, now appearing as unstageable PI. Selected cannot obtain a good seal, and pt is not at risk for incontinence. Would consider: updating the WOCN with picture and message/call. In the interrim, would consider triad in dime thickness applied over wound base left OTA. Surgical consult may be require, and if not able to place foam dressing may require advanced dressing placement.

- ☐ return to beginning of form
- ☐ end of form

42

sDTPI wet (evolving) over the buttocks, now appearing as unstageable PI. Selected cannot obtain a good seal, and pt is at risk for incontinence. Would consider: updating the WOCN with picture and message/call. In the interrim, would consider triad in dime thickness applied over wound base left OTA. Surgical consult may be require, and if not able to place foam dressing may require advanced dressing placement. offload/turn per protocol and guidelines

- ☐ return to beginning of form
- ☐ end of form

43

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an otherwise flat and dry. please select from the following options

- ☐ I can obtain a good seal with a allevyn foam dressing, pt is not at risk for incontinence.
- ☐ I can obtain a good seal with a allevyn foam dressing, pt is at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is not at risk for incontinence
- ☐ I cannot obtain a good seal with a foam dressing, pt is at risk for incontinence

44

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an otherwise flat and dry. Pt is NOT at risk for incontinence, and CAN obtain a good seal. consider the following: update WOCN team with picture and message/call. Consider: painting the area with cation, apply contact layer of mepitel one/adaptic cut to size, secured with allevyn foam dressing and changed every other day, the dressing partially peeled back to examine Q shift. continue offloading and turning measures per protocols/guideliens.

- ☐ return to beginning of form
- ☐ end of form

45

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an otherwise flat and dry. Pt IS at risk for incontinence, and CAN obtain a good seal. consider the following: update WOCN team with picture and message/call. Consider: painting the area with caviton, apply contact layer of mepitel one/adaptic cut to size, secured with allevyn foam dressing and changed every other day, the dressing partially peeled back to examine Q shift. continue offloading and turning measures per protocols/guideliens.

- ☐ return to beginning of form
- ☐ end of form

46

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an otherwise flat and dry. Pt is NOT at risk for incontinence, and CANNOT obtain a good seal. consider the following: update WOCN team with picture and message/call. Consider: critic aid ointment BID and with each episode of incontinence/cleaning. The area remains intact and may continue to be stable. offload and turn per protocols/guidelines.

- ☐ return to beginning of form
- ☐ end of form

47

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now an otherwise flat and dry. Pt IS at risk for incontinence, and CANNOT obtain a good seal. consider the following: update WOCN team with picture and message/call. Consider: baza BID and with each episode of cleaning. Continue turning and offloading per protocol/guidelines

- ☐ return to beginning of form
- ☐ end of form

48

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now presenting as partially opening wet wound, but cannot discern the wound stage or the skin has not completely broken. If able to obtain a good seal, apply triad and foam dressing once daily. If not able to obtain a good seal, apply triad and leave open to air. Call/message WOCN with updates, and picture. continue turning and offloading per protocols and guidelines.

- ☐ return to beginning of form
- ☐ end of form

49

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) buttocks selected, now presenting as skin fully removed, but the tissue is either pink OR still purple in some areas. the deep discoloration means that that tissue is expected to likely turn into eschar at a later date. Consider: messaging/calling WOCN team with picture for updated recs. While surgical debridement may be needed in the future, it may not be necessary right this moment. If able to obtain good seal, apply triad and cover with foam changed once daily. If not able to obtain good seal, apply triad and leave OTA.

☐ return to beginning of form

☐ end of form

50

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER limbs/torso/back selected. Which more resembles the current stage of your wound?



☐ now becoming an unstageable



☐ remains flat and otherwise dry



☐ partial opening, but cannot definitively re-stage



☐ skin loss is present, but cannot effectively re-stage, and/OR tissue remains deep purple

51

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER limbs/torso/back selected now becoming an unstageable. Consider messaging WOCN or calling for updated recs. A surgical consult MAY be required, and then beginning santyl dressing once daily. However, if the unstageable is over a limb where the pt has hx of PAD/LEVD, other considerations may need to be taken. In the interrim while waiting for updates, continue nonadherent contact layer secured by foam dressing.

☐ return to beginning of form

☐ end of form

52

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER limbs/torso/back selected now becoming an partially open, but cannot definitively restage. The purple discoloration means the tissue may continue to evolve into necrosis. There is no clear slough or eschar to begin debridement. Would consider using triad and foam dressing changed once daily, and update WOCN via message or call for clarification.

☐ return to beginning of form

☐ end of form

53

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER limbs/torso/back selected now showing as a flat dry wound bed, that is not evolving. Would consider cavilon, continued non adherent contact layer (mepitel one/adaptic) secured by foam dressing changed every other day and peeled slightly back for daily inspection.

☐ return to beginning of form

☐ end of form

54

sDTPI wet (evolving, skin breaking or opening, seeing progression past the epidermis) selected OVER limbs/torso/back selected now showing as a skin loss, unclear if the wound bed appears pink/purple, would consider: triad and foam dressing once daily. triad is moreso a "touch up job" do not scrub off the triad unless otherwise indicated. Consider messaging or calling your WOCN for an update.

☐ return to beginning of form

☐ end of form

55

Evolving DTI over limb, over the heel or foot specifically, consider consulting podiatry, then WOCN, and for the time being paint the area with betadine OR cavilon, secure with NON ADHERENT gauze OR adaptic/mepitel one silicone, secured by kerlix/bulkee and offloaded appropriately.

☐ return to beginning of form

☐ end of form

Skin Tear Assessment

Determine location of your wound (Skin tear)

56

Is your skin tear over an extremity/limb (arms/legs), or other body surface?

- ☐ arms/legs
- ☐ other body surface

57

Skin tear over other body surface (torso, perineal, buttocks), is the skin flap present?

NO SKIN LOSS

(Flap tear which can be repositioned to cover the wound bed).

PARTIAL SKIN LOSS

(Skin loss which cannot be repositioned to cover the wound bed).

TOTAL SKIN LOSS

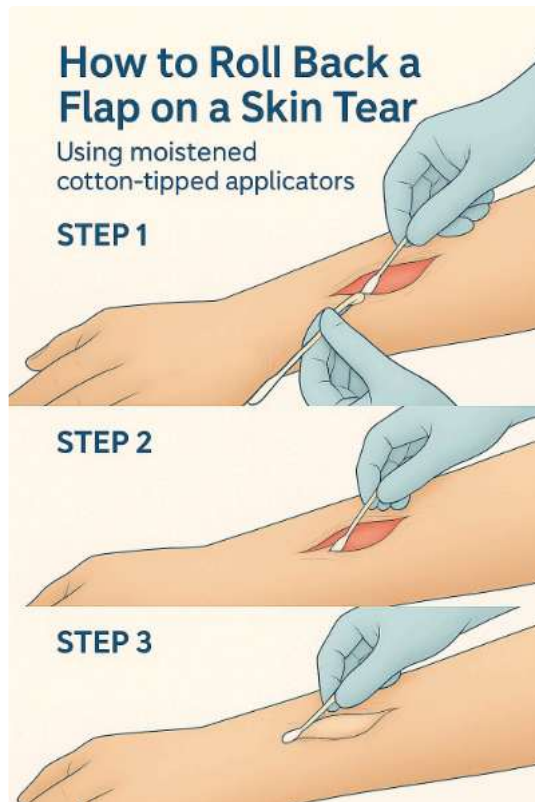
(Total skin loss exposing the entire wound bed).



- ☐ Yes
- ☐ No

58

Skin tear over other body surface (torso, perineal, buttocks), with skin flap present, can you approximate the flap?



☐ Yes

☐ No

59

Skin tear over the body surface (torso, perineal, buttocks), with skin flap present that can be approximated, what level is the drainage?

☐ minimal

☐ moderate

60

Skin tear over the body surface with approximated flap that has moderate drainage: is this located on torso, head or buttocks/perineal region?

- ☐ torso
- ☐ head
- ☐ perineal region

61

Skin tear over the body surface with approximated flap that has minor drainage: is this located on torso, head or buttocks/perineal region?

- ☐ torso
- ☐ head
- ☐ perineal region

Skin tear over the body surface with approximated flap that has minor drainage: located on the torso: would consider gentle saline cleanse, use moistened cotton tip applicator to roll back the flap, apply cavilon, and dress with hydrocolloid comfeel dressing changed once weekly.

	Hydration:	Explanation:	Cover With:	Explanation:
SHALLOW DRY WOUNDS		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover.

- ☐ return to beginning of form
- ☐ end of form

Skin tear over the body surface with approximated flap that has minor drainage: located on the perineal:
IF ABLE TO OBTAIN A GOOD SEAL AND AVOID INCONTINENCE-would consider gentle saline cleanse, use moistened cotton tip applicator to roll back the flap, apply cavilon, use xeroform cut to size and secure with allevyn foam dressing changed once daily. IF NOT ABLE TO AVOID INCONTINENCE AND OR NOT ABLE TO OBTAIN GOOD SEAL WITH DRESSING-apply triad open to air once daily. consider a waffle cushion in bed and out of bed to chair.

	Hydration:	Explanation:	Cover With:	Explanation:
SHALLOW DRY WOUNDS		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PF	N/A	N/A
SHALLOW WET WOUNDS		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PF	N/A	Serves as contact layer and cover

- ☐ return to beginning of form
- ☐ end of form

For skin tear over the head please consult plastics or dermatology. For additional consideration, send a picture and description to your WOCN, and contact over teams/email.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover

☐ return to beginning of form

☐ end of form

Skin tear over torso with moderate drainage and flap that can be approximated, would consider: gentle cleanse with saline, use moistened cotton tip applicator to roll back the flap, paint the area with cavilon, cover with non adherent contact layer such as adaptic or mepitel one, then secure an absorptive dressing such as aquacell or optifoam, secure island/telfa dressing and change every other day.

	Hydration:	Explanation:	Cover With:	Explanation:
SHALLOW DRY WOUNDS		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than wovens. Wet before removing.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover.

- ☐ return to beginning of form
- ☐ end of form

Skin tear over torso with minimal drainage and flap that can be approximated, would consider: gentle cleanse with saline, use moistened cotton tip applicator to roll back the flap, paint the area with cavilon, cover with hydrocolloid dressing

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover

- ☐ return to beginning of form
- ☐ end of form

67

Skin tear over buttocks/perineal with minor drainage, with a flap present, consider: for wounds that are able to be isolated from incontinence, consider cavilon and hydrocolloid dressing changed once daily. for wounds that cannot be isolated for further irritation, consider triad (zinc/oxide demethicone) daily.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover

- ☐ return to beginning of form
- ☐ end of form

skin tear over the buttocks/perineal with moderate drainage, please consider consulting WOCN.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover.

☐ return to beginning of form

☐ end of form

69

Skin tear over the torso with minimal drainage, a flap that CANNOT be approximated are is not present, would consider: gentle cleanse with saline. if <3x3cm consider hydrocolloid dressing changed once weekly. if >3x3cm, consider xeroform cut to size secured by telfa island dressing and changed once daily.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than wovens. Wet before removing.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover.

- ☐ return to beginning of form
- ☐ end of form

70

Skin tear over other body surface (torso, perineal, buttocks), with skin flap NOT present, what level is the drainage?

- ☐ minimal
- ☐ moderate

71

Skin tear over the arm/legs: is there a skin flap present?

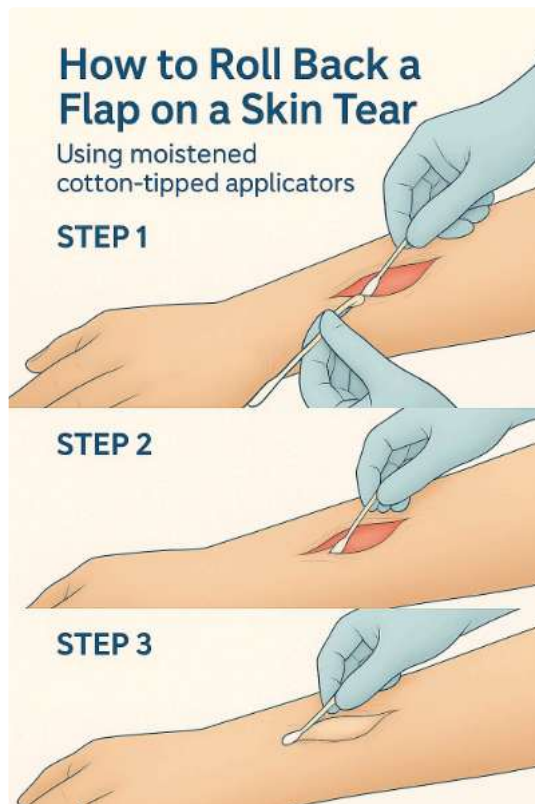


☐ Yes

☐ No

72

Skin tear over the arms/legs with a flap: Are you able to approximate the flap?



☐ Yes

☐ No

73

Skin tear over the arms/legs without a flap: What level is the drainage?

☐ moderate

☐ minimal

74

Skin tear over arms/legs and able to approximate flap: What level is the drainage?

☐ minimal

☐ moderate

For a wound with minimal drainage, a skin flap that can be approximated, over a limb, would consider a gentle saline cleanse, used moistened cotton tipped applicators to approximate the flap, steri strips (if needed to secure the flap), painted with cavilon, then apply a hydrocolloid dressing (comfeel). changed weekly. Label the dressing in direction opposite of flap to avoid damage with dressing changes.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds		Same as above
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds		Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI	N/A	N/A
SHALLOW WET WOUNDS	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin Consider skin tears with flaps, stage 2 PI	N/A	Serves as contact layer and cover

- ☐ return to beginning of form
- ☐ end of form

For a wound with moderate drainage, a skin flap that can be approximated, over a limb, would consider: gentle cleanse with saline, use moistened cotton tip applicator to roll back the flap, paint the area with cavilon, cover with non adherent contact layer such as adaptic or mepitel one, then secure an absorptive dressing such as aquacell or optifoam, secure with abd pad and kerlix changed every other day.

	Hydration:	Explanation:	Cover With:	Explanation:
SHALLOW DRY WOUNDS		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds.		Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI.	N/A	N/A
SHALLOW WET WOUNDS		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.		Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI.	N/A	Serves as contact layer and cover.

- ☐ return to beginning of
- ☐ end of form

77

For a wound with minimal drainage, a skin flap present that could not be approximated and/or is not present, would consider: assess size of the wound. If small enough, (i.e. less than 3x3cm), would consider a gentle saline cleanse, cavilon, and a hydrocolloid dressing. If large and at risk for infection, would consider cavilon to the periwound, drape with single layer xeroform, then secure with non woven gauze and kerlix changed once daily.

	Hydration:	Explanation:	Cover With:	Explanation:
SHALLOW DRY WOUNDS		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PI	N/A	N/A
SHALLOW WET WOUNDS		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PI	N/A	Serves as contact layer and cover

☐ return to beginning of form'

☐ end of form

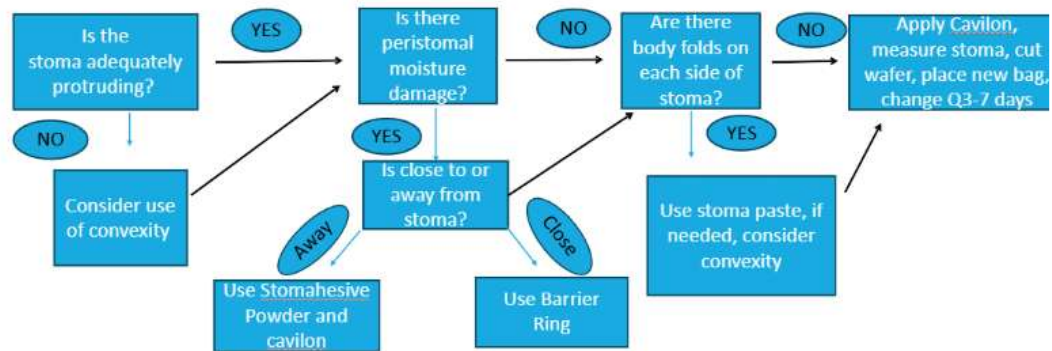
For a wound with moderate drainage, a skin flap present that could not be approximated and/or is not present, over a limb, would consider: saline cleanse, apply layer of adaptic or mepitel one, then cover with layer of aquacell extra. Cover additionally with non woven gauze 4x4 OR an abd pad, kerlix, change every other day. Once the drainage is controlled or exudate is minimal, consider replacing with hydrocolloid comfeel dressing changed weekly.

SHALLOW DRY WOUNDS	Hydration:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. May increase wound exudate initially. Draws moisture from deep within wound bed. Dehydrates and kills bacteria. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive. If agitated wound bed, use contact layer
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above
		Promotes autolytic debridement. May increase wound exudate initially. Donates moisture to wound base. Consider dry wounds	  	Same as above.
SHALLOW WET WOUNDS		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Consider skin tears with flaps, stage 2 PF	N/A	N/A
	Dry:	Explanation:	Cover With:	Explanation:
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider for infected wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Draws moisture to cover layer, off the wound bed. Gels on contact with moisture for traumatic changes. Consider moderate drainage wounds	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Non-woven, low linting, less adherence to wound base than woven. Wet before removing.	  	Cavilon to protect peri-wound from adhesive tears, and further moisture damage. Tegaderm for closer seal, foam dressing for gentle adhesive.
		Promotes autolytic debridement. Contains heat and moisture to hydrate wound bed. Pulls excess moisture off skin. Consider skin tears with flaps, stage 2 PF	N/A	Serves as contact layer and cover

- ☐ return to beginning of form
- ☐ to end of form

Ostomy Assessment

Issue To Intervention: Treat What's There (Read From L→R)



79

Does the pt have one of the following: ileostomy, colostomy, urostomy? *



☐ Yes

☐ No

80

You selected no, please consider WOCN consult, reach out by teams or "wound educator" over vocera, or re-check with surgical team if pt has an ostomy. *

☐ end this form

☐ return to beginning of this form

81

When changing an ostomy appliance, you need to identify different aspects-stoma size, shape, color, and skin around the area.

1. stoma size: within each box of skin wafer box (2 piece) is a circle stoma measuring guide. if needing an oval measuring guide, reach out to WOCN. 2.Shape: can be round, oval, elliptical, appear different when stomach is folded or flat. 3. Color: post op darker red, chronic appears more pink, *hypoperfusion* appears black! Notify MD.4. Skin around the area: is it damaged, intact, are the abdomen's contours creating folds, dips, or areas contributing to leakage? *

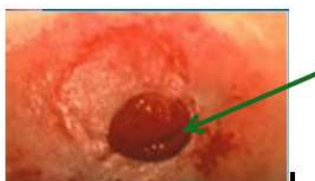
☐ continue onto next phase

82

We are dealing with a patient that has an ostomy. Now, where is the "os" or opening of the stoma ? *



☐ center



☐ leaning off to the side



☐ opens towards the skin level

83

You selected os opens to the center, meaning the stool will exit from the middle of stoma, and generally is easier to direct this drainage into the pouch.

Now, assess whether the skin around the stoma appears protruding above the skin level, or is retracted/flushed with the skin *



☐ above skin level



☐ retracted / flushed / buried by skin folds

84

You selected os opens off to the side, meaning the stool will exit from the opening of this stoma towards the direction it faces.

This can prove challenging if you have a short/retracted/buried stoma, or a longer stoma that tilts at an odd angle.

The most important thing is to be sure to never cover the opening of the stoma, otherwise you will risk leakage. Consider cutting slightly larger (1-2mm) larger near this portion of the stoma.

Now, assess whether the skin around the stoma appears protruding above the skin level, or is retracted/flushed with the skin *



☐ above skin level



☐ retracted / flushed / buried by skin folds

85

You selected os opens near or at the skin level, meaning the stool will exit from the opening of this stoma towards the direction the stoma faces, and can often end up going underneath your dressing.

This can prove challenging if you have a short/retracted/buried stoma, or a longer stoma that tilts at an odd angle. The most important thing is to be sure to never cover the opening of the stoma, otherwise you will risk leakage. Consider cutting slightly larger (1-2mm) larger near this portion of the stoma.

Now, assess whether the skin around the stoma appears protruding above the skin level, or is retracted/flushed with the skin *



☐ above skin level



☐ retracted / flushed / buried by skin folds

86

You selected that the stoma is protruding above skin level, and is not blocked by skin folds. Now is that skin around the stoma intact, or is it broken/raw/damaged? *

- ☐ skin is intact
- ☐ skin is damaged

87

You selected an ostomy with a protruding stoma, and the skin around the stoma is intact/WDL. Cleanse the area sterile water, pat dry. Apply cavilon no sting barrier skin prep, measure your stoma to size and shape, cut your appliance, and place FLAT skin wafer on patient. Be sure to pull the skin taut before laying down adhesive. Date the skin wafer for next change. *

- ☐ return to the beginning of the form
- ☐ end the form

88

You selected that the stoma is retracted/flushed/flat at or below the skin level, or a stoma which may be protruding but the skin folds around it are almost "burying it like a moat". Now is that skin around the stoma intact, or is it broken/raw/damaged? *

- ☐ skin is intact
- ☐ skin is damaged

89

You selected a stoma that is retracted/flushed/flat/buried, and the skin is intact.

1. Cleanse the area sterile water, pat dry. 2. Apply cavilon no sting barrier skin prep3. measure your stoma to size and shape, cut your appliance, and place CONVEX wafer on patient. Be sure to pull the skin taut before laying down adhesive. Date the skin wafer for next change.

*

- ☐ return to beginning of form
- ☐ end the form

90

You selected a protruding stoma, where the skin around the stoma is damaged. Select the level or similar kind of moisture damage you see on your patient: *



- ☐ Minor moisture damage. Rash may be red/pink, centered around the stoma, or spread farther away from the stoma. scant skin breakdown, otherwise dry.



- ☐ Moderate moisture damage. skin is indurated (deep red rash), and skin may be peeling or bleeding.



- ☐ Severe moisture damage. Skin is raw, indurated, and continuously weeping despite cleansing or skin prep (cavilon)

91

You selected a retracted/flushed/buried stoma, where the skin around the stoma is damaged. Select the level or similar kind of moisture damage you see on your patient: *



- ☐ Minor moisture damage. Rash may be red/pink, centered around the stoma, or spread farther away from the stoma. scant skin breakdown, otherwise dry.



- ☐ Moderate moisture damage. skin is indurated (deep red rash), and skin may be peeling or bleeding.



- ☐ Severe moisture damage. Skin is raw, indurated, and continuously weeping despite cleansing or skin prep (cavilon)

92

You selected a protruding stoma, with the skin having minor moisture damage. A rash may be present, but is controlled, isolated, and can be managed with more or less intact skin.

1. Cleanse the area with sterile water
2. apply cavilon no sting barrier film to the periwound, consider using an ostomy barrier ring if leakage persists
3. otherwise, measure your stoma size and shape, and apply new ostomy pouch with a FLAT skin wafer/sticker. *

- ☐ end this form
- ☐ return to beginning of form

93

You selected a retracted/flushed/buried stoma, with the skin having minor moisture damage. A rash may be present, but is controlled, isolated, and can be managed with more or less intact skin.

1. Cleanse the area with sterile water
2. apply cavilon no sting barrier film to the periwound, consider using an ostomy barrier ring if leakage persists
3. otherwise, measure your stoma size and shape, and apply new ostomy pouch with a CONVEX skin wafer/sticker. *

- ☐ end this form
- ☐ return to beginning of form

94

You selected a protruding stoma, with the skin having moderate moisture damage. A rash may be present, with wet raw broken skin making it difficult to place a dressing. Please update your WOCN, and send a picture of the wound

1. Cleanse the area gently with sterile water, pat dry.
2. Apply stomahesive POWDER in light-fine layer (less is more).
3. Gently rub the powder into the damaged skin, dusting off excess.
4. Use cavilon no sting barrier film to soak the powder, and it will form a sticky crust.
5. Consider using an ostomy barrier ring if leakage persists otherwise, measure your stoma size and shape, and apply new ostomy pouch with a FLAT skin wafer/sticker. *

- ☐ end this form
- ☐ return to beginning of form

95

You selected a retracted/flushed/buried stoma, with the skin having moderate moisture damage. A rash may be present, with wet raw broken skin making it difficult to place a dressing. Please update your WOCN with picture and scenario.

1. Cleanse the area gently with sterile water, pat dry.
2. Apply stomahesive POWDER in light-fine layer (less is more). Gently rub the powder into the damaged skin, dusting off excess.
3. Use cavilon no sting barrier film to soak the powder, and it will form a sticky crust.
4. Consider using an ostomy barrier ring if leakage persists otherwise, measure your stoma size and shape, and apply new ostomy pouch with a CONVEX skin wafer/sticker. *

- ☐ end this form
- ☐ return to beginning of form

96

You selected a protruding stoma, with the skin having severe moisture damage. A rash may be present, the skin is raw, indurated, and continuously weeping despite cleansing or skin prep (cavilon). We need to obtain a dry clean surface of skin to work with however. One option is to apply a dressing "skin substitute" to cover the damaged areas which then makes a clean dry surface to start building a new ostomy dressing on.

1. cleanse with sterile water, pat dry. 2. Apply stomahesive POWDER in light-fine layer (less is more). Gently rub the powder into the damaged skin, dusting off excess. Use cavilon no sting barrier film to soak the powder, and it will form a sticky crust. 3. Use a coloplast brava protector sheet to cover the damaged areas of skin. Be sure to cut a "hole" slightly larger then the size and shape of your stoma, and lay over it
4. Be sure the dressing is laid down flat and the skin is pulled somewhat taunt before laying down adhesive.
5. Now, we can begin building a new ostomy dressing over this "new skin layer". Consider using an eakins ring to help ensure your new "skin layer" does not have stool leaking under the dressing, by placing the ring as close around the stoma to intact skin.
6. Measure and cut a FLAT ostomy wafer for your patient to their size and shape, and connect to pouch. Date for next dressing change. Please update your WOCN on this patient. *



- ☐ end this form
- ☐ return to beginning.

97

You selected a retracted/flushed/buried stoma, with the skin having severe moisture damage. A rash may be present, the skin is raw, indurated, and continuously weeping despite cleansing or skin prep (cavilon). We need to obtain a dry clean surface of skin to work with however. One option is to apply a dressing "skin substitute" to cover the damaged areas which then makes a clean dry surface to start building a new ostomy dressing on.

1. cleanse with sterile water, pat dry. 2. Apply stomahesive POWDER in light-fine layer (less is more). Gently rub the powder into the damaged skin, dusting off excess. Use cavilon no sting barrier film to soak the powder, and it will form a sticky crust. 3. Use a coloplast brava protector sheet to cover the damaged areas of skin. Be sure to cut a "hole" slightly larger than the size and shape of your stoma, and lay over it
4. Be sure the dressing is laid down flat and the skin is pulled somewhat taut before laying down adhesive.
5. Now, we can begin building a new ostomy dressing over this "new skin layer". Consider using an eakins ring to help ensure your new "skin layer" does not have stool leaking under the dressing, by placing the ring as close around the stoma to intact skin.
6. Measure and cut a CONVEX ostomy wafer for your patient to their size and shape, and connect to pouch. Date for next dressing change. Please update your WOCN on this patient. *



- ☐ end this form
- ☐ return to beginning.

Moisture Dermatitis-Currently in Development

MASD Type	Example		Dressing
IAD			Intact: moisture barrier Broken: Triad
ITD (Intertrigo)			Intact: Powder vs Interdry Broken: Triad over rupture, dressing, then Interdry
Peristomal moisture dermatitis			Clean, pat dry, stomahesive powder (less is more), soak with Cavilon
Periwound moisture dermatitis			Minor: Cavilon Major: Triad in thin layer at wound edge
Moisture Management Product	Explanation		Application
Baza 	Zinc oxide centered wound base, great for heavily denuded macerated skin.		Cleanse area. Apply BID and with each episode of incontinence
Critic-Aid Clear 	Dimethicone based petroleum ointment, hydrates, protects. Great for minor to moderate skin damage.		Cleanse area Apply BID and with each episode of incontinence
Triad 	Zinc oxide based wound paste that absorbs exudate and is sting free on raw skin. Meant for heavily damaged skin, and partial thickness wounds. Can be left open to air or covered.		Open to air: BID and PRN Covered by dressing: QD and PRN

Ending

Please provide your feedback, which is imperative for continuing this or a service like this in the future!

98

What is your role? *

☐ provider

☐ RN

☐ NA

99

You are at the end of this form. Thank you for participating and understanding that this is still in development. Look forward to future developments! Please select from the following: *

	Highly disagree	Disagree	Neutral	Agree	Highly agree
This product was easy to use	☐	☐	☐	☐	☐
I believe this product will help reduce wait times for wound consults	☐	☐	☐	☐	☐
This product helps me to streamline care for my patients	☐	☐	☐	☐	☐
This product helps to reinforce my unit education	☐	☐	☐	☐	☐
This product may help me form a practice change	☐	☐	☐	☐	☐
I knew what these dressings were before using this product	☐	☐	☐	☐	☐
I knew how to treat these wounds before using this product	☐	☐	☐	☐	☐
Using this tool means we do not need a WOCN	☐	☐	☐	☐	☐

100

Any feedback? Any issues? Anything you'd like to see changed or added? Thank you! *

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